

MED-R REAL PUBMED REPORT

date: 2026-02-12

primary field: oncology

sub field: immunotherapy

Gene: EGFR

Anchor positive papers: 15

Target field: cardiology

Creep positive papers: 0

Score: 15.0

Proposed experiments:

- experiment 1 (triple-intersection pilot): identify 5 patients with oncology + cardiology comorbidity under immunotherapy-relevant treatment context; track pre/post markers; early-kill if no directional change in 2–4 weeks
- experiment 2 (retrospective signal): query existing records/literature in cardiology for EGFR associations under immunotherapy; early-kill if effect is null or contradictory
- experiment 3 (biomarker correlation): measure EGFR proxy/marker vs clinical endpoints in cardiology cohorts receiving immunotherapy-related interventions; early-kill if correlation < threshold
- experiment 4 (mechanism proxy assay): run a cheap in vitro assay relevant to cardiology with perturbation of EGFR under immunotherapy-like conditions; early-kill if no response vs control
- experiment 5 (replication stress test): repeat the strongest assay with 2 independent datasets/conditions; early-kill if signal is not reproducible

Target field: neurology

Creep positive papers: 0

Score: 15.0

Proposed experiments:

- experiment 1 (triple-intersection pilot): identify 5 patients with oncology + neurology comorbidity under immunotherapy-relevant treatment context; track pre/post markers; early-kill if no directional change in 2–4 weeks
- experiment 2 (retrospective signal): query existing records/literature in neurology for EGFR associations under immunotherapy; early-kill if effect is null or contradictory
- experiment 3 (biomarker correlation): measure EGFR proxy/marker vs clinical endpoints in neurology cohorts receiving immunotherapy-related interventions; early-kill if correlation < threshold
- experiment 4 (mechanism proxy assay): run a cheap in vitro assay relevant to neurology with perturbation of EGFR under immunotherapy-like conditions; early-kill if no response

vs control

- experiment 5 (replication stress test): repeat the strongest assay with 2 independent datasets/conditions; early-kill if signal is not reproducible

Target field: pulmonology

Creep positive papers: 0

Score: 15.0

Proposed experiments:

- experiment 1 (triple-intersection pilot): identify 5 patients with oncology + pulmonology comorbidity under immunotherapy-relevant treatment context; track pre/post markers; early-kill if no directional change in 2–4 weeks
- experiment 2 (retrospective signal): query existing records/literature in pulmonology for EGFR associations under immunotherapy; early-kill if effect is null or contradictory
- experiment 3 (biomarker correlation): measure EGFR proxy/marker vs clinical endpoints in pulmonology cohorts receiving immunotherapy-related interventions; early-kill if correlation < threshold
- experiment 4 (mechanism proxy assay): run a cheap in vitro assay relevant to pulmonology with perturbation of EGFR under immunotherapy-like conditions; early-kill if no response vs control
- experiment 5 (replication stress test): repeat the strongest assay with 2 independent datasets/conditions; early-kill if signal is not reproducible

Target field: immunology

Creep positive papers: 1

Score: 7.5

Proposed experiments:

- experiment 1 (triple-intersection pilot): identify 5 patients with oncology + immunology comorbidity under immunotherapy-relevant treatment context; track pre/post markers; early-kill if no directional change in 2–4 weeks
- experiment 2 (retrospective signal): query existing records/literature in immunology for EGFR associations under immunotherapy; early-kill if effect is null or contradictory
- experiment 3 (biomarker correlation): measure EGFR proxy/marker vs clinical endpoints in immunology cohorts receiving immunotherapy-related interventions; early-kill if correlation < threshold
- experiment 4 (mechanism proxy assay): run a cheap in vitro assay relevant to immunology with perturbation of EGFR under immunotherapy-like conditions; early-kill if no response vs control
- experiment 5 (replication stress test): repeat the strongest assay with 2 independent datasets/conditions; early-kill if signal is not reproducible

Gene: HER2

Anchor positive papers: 11

Target field: cardiology

Creep positive papers: 0

Score: 11.0

Proposed experiments:

- experiment 1 (triple-intersection pilot): identify 5 patients with oncology + cardiology comorbidity under immunotherapy-relevant treatment context; track pre/post markers; early-kill if no directional change in 2–4 weeks
- experiment 2 (retrospective signal): query existing records/literature in cardiology for HER2 associations under immunotherapy; early-kill if effect is null or contradictory
- experiment 3 (biomarker correlation): measure HER2 proxy/marker vs clinical endpoints in cardiology cohorts receiving immunotherapy-related interventions; early-kill if correlation < threshold
- experiment 4 (mechanism proxy assay): run a cheap in vitro assay relevant to cardiology with perturbation of HER2 under immunotherapy-like conditions; early-kill if no response vs control
- experiment 5 (replication stress test): repeat the strongest assay with 2 independent datasets/conditions; early-kill if signal is not reproducible

Target field: neurology

Creep positive papers: 0

Score: 11.0

Proposed experiments:

- experiment 1 (triple-intersection pilot): identify 5 patients with oncology + neurology comorbidity under immunotherapy-relevant treatment context; track pre/post markers; early-kill if no directional change in 2–4 weeks
- experiment 2 (retrospective signal): query existing records/literature in neurology for HER2 associations under immunotherapy; early-kill if effect is null or contradictory
- experiment 3 (biomarker correlation): measure HER2 proxy/marker vs clinical endpoints in neurology cohorts receiving immunotherapy-related interventions; early-kill if correlation < threshold
- experiment 4 (mechanism proxy assay): run a cheap in vitro assay relevant to neurology with perturbation of HER2 under immunotherapy-like conditions; early-kill if no response vs control
- experiment 5 (replication stress test): repeat the strongest assay with 2 independent datasets/conditions; early-kill if signal is not reproducible

Target field: metabolic

Creep positive papers: 1

Score: 5.5

Proposed experiments:

- experiment 1 (triple-intersection pilot): identify 5 patients with oncology + metabolic comorbidity under immunotherapy-relevant treatment context; track pre/post markers; early-kill if no directional change in 2–4 weeks
- experiment 2 (retrospective signal): query existing records/literature in metabolic for HER2 associations under immunotherapy; early-kill if effect is null or contradictory
- experiment 3 (biomarker correlation): measure HER2 proxy/marker vs clinical endpoints in metabolic cohorts receiving immunotherapy-related interventions; early-kill if correlation < threshold
- experiment 4 (mechanism proxy assay): run a cheap in vitro assay relevant to metabolic with perturbation of HER2 under immunotherapy-like conditions; early-kill if no response vs control
- experiment 5 (replication stress test): repeat the strongest assay with 2 independent datasets/conditions; early-kill if signal is not reproducible

Target field: pulmonology

Creep positive papers: 0

Score: 11.0

Proposed experiments:

- experiment 1 (triple-intersection pilot): identify 5 patients with oncology + pulmonology comorbidity under immunotherapy-relevant treatment context; track pre/post markers; early-kill if no directional change in 2–4 weeks
- experiment 2 (retrospective signal): query existing records/literature in pulmonology for HER2 associations under immunotherapy; early-kill if effect is null or contradictory
- experiment 3 (biomarker correlation): measure HER2 proxy/marker vs clinical endpoints in pulmonology cohorts receiving immunotherapy-related interventions; early-kill if correlation < threshold
- experiment 4 (mechanism proxy assay): run a cheap in vitro assay relevant to pulmonology with perturbation of HER2 under immunotherapy-like conditions; early-kill if no response vs control
- experiment 5 (replication stress test): repeat the strongest assay with 2 independent datasets/conditions; early-kill if signal is not reproducible

Gene: MSI

Anchor positive papers: 9

Target field: cardiology

Creep positive papers: 0

Score: 9.0

Proposed experiments:

- experiment 1 (triple-intersection pilot): identify 5 patients with oncology + cardiology comorbidity under immunotherapy-relevant treatment context; track pre/post markers; early-kill if no directional change in 2–4 weeks
- experiment 2 (retrospective signal): query existing records/literature in cardiology for MSI associations under immunotherapy; early-kill if effect is null or contradictory
- experiment 3 (biomarker correlation): measure MSI proxy/marker vs clinical endpoints in cardiology cohorts receiving immunotherapy-related interventions; early-kill if correlation < threshold
- experiment 4 (mechanism proxy assay): run a cheap in vitro assay relevant to cardiology with perturbation of MSI under immunotherapy-like conditions; early-kill if no response vs control
- experiment 5 (replication stress test): repeat the strongest assay with 2 independent datasets/conditions; early-kill if signal is not reproducible

Target field: neurology

Creep positive papers: 1

Score: 4.5

Proposed experiments:

- experiment 1 (triple-intersection pilot): identify 5 patients with oncology + neurology comorbidity under immunotherapy-relevant treatment context; track pre/post markers; early-kill if no directional change in 2–4 weeks
- experiment 2 (retrospective signal): query existing records/literature in neurology for MSI associations under immunotherapy; early-kill if effect is null or contradictory
- experiment 3 (biomarker correlation): measure MSI proxy/marker vs clinical endpoints in neurology cohorts receiving immunotherapy-related interventions; early-kill if correlation < threshold
- experiment 4 (mechanism proxy assay): run a cheap in vitro assay relevant to neurology with perturbation of MSI under immunotherapy-like conditions; early-kill if no response vs control
- experiment 5 (replication stress test): repeat the strongest assay with 2 independent datasets/conditions; early-kill if signal is not reproducible

Target field: pulmonology

Creep positive papers: 0

Score: 9.0

Proposed experiments:

- experiment 1 (triple-intersection pilot): identify 5 patients with oncology + pulmonology comorbidity under immunotherapy-relevant treatment context; track pre/post markers; early-kill if no directional change in 2–4 weeks
- experiment 2 (retrospective signal): query existing records/literature in pulmonology for MSI associations under immunotherapy; early-kill if effect is null or contradictory
- experiment 3 (biomarker correlation): measure MSI proxy/marker vs clinical endpoints in

pulmonology cohorts receiving immunotherapy-related interventions; early-kill if correlation < threshold

- experiment 4 (mechanism proxy assay): run a cheap in vitro assay relevant to pulmonology with perturbation of MSI under immunotherapy-like conditions; early-kill if no response vs control
- experiment 5 (replication stress test): repeat the strongest assay with 2 independent datasets/conditions; early-kill if signal is not reproducible

Gene: CTLA

Anchor positive papers: 9

Target field: cardiology

Creep positive papers: 1

Score: 4.5

Proposed experiments:

- experiment 1 (triple-intersection pilot): identify 5 patients with oncology + cardiology comorbidity under immunotherapy-relevant treatment context; track pre/post markers; early-kill if no directional change in 2–4 weeks
- experiment 2 (retrospective signal): query existing records/literature in cardiology for CTLA associations under immunotherapy; early-kill if effect is null or contradictory
- experiment 3 (biomarker correlation): measure CTLA proxy/marker vs clinical endpoints in cardiology cohorts receiving immunotherapy-related interventions; early-kill if correlation < threshold
- experiment 4 (mechanism proxy assay): run a cheap in vitro assay relevant to cardiology with perturbation of CTLA under immunotherapy-like conditions; early-kill if no response vs control
- experiment 5 (replication stress test): repeat the strongest assay with 2 independent datasets/conditions; early-kill if signal is not reproducible

Target field: neurology

Creep positive papers: 1

Score: 4.5

Proposed experiments:

- experiment 1 (triple-intersection pilot): identify 5 patients with oncology + neurology comorbidity under immunotherapy-relevant treatment context; track pre/post markers; early-kill if no directional change in 2–4 weeks
- experiment 2 (retrospective signal): query existing records/literature in neurology for CTLA associations under immunotherapy; early-kill if effect is null or contradictory
- experiment 3 (biomarker correlation): measure CTLA proxy/marker vs clinical endpoints in neurology cohorts receiving immunotherapy-related interventions; early-kill if correlation < threshold

- experiment 4 (mechanism proxy assay): run a cheap in vitro assay relevant to neurology with perturbation of CTLA under immunotherapy-like conditions; early-kill if no response vs control
- experiment 5 (replication stress test): repeat the strongest assay with 2 independent datasets/conditions; early-kill if signal is not reproducible

Target field: pulmonology

Creep positive papers: 1

Score: 4.5

Proposed experiments:

- experiment 1 (triple-intersection pilot): identify 5 patients with oncology + pulmonology comorbidity under immunotherapy-relevant treatment context; track pre/post markers; early-kill if no directional change in 2–4 weeks
- experiment 2 (retrospective signal): query existing records/literature in pulmonology for CTLA associations under immunotherapy; early-kill if effect is null or contradictory
- experiment 3 (biomarker correlation): measure CTLA proxy/marker vs clinical endpoints in pulmonology cohorts receiving immunotherapy-related interventions; early-kill if correlation < threshold
- experiment 4 (mechanism proxy assay): run a cheap in vitro assay relevant to pulmonology with perturbation of CTLA under immunotherapy-like conditions; early-kill if no response vs control
- experiment 5 (replication stress test): repeat the strongest assay with 2 independent datasets/conditions; early-kill if signal is not reproducible

Gene: ECOG

Anchor positive papers: 9

Target field: cardiology

Creep positive papers: 0

Score: 9.0

Proposed experiments:

- experiment 1 (triple-intersection pilot): identify 5 patients with oncology + cardiology comorbidity under immunotherapy-relevant treatment context; track pre/post markers; early-kill if no directional change in 2–4 weeks
- experiment 2 (retrospective signal): query existing records/literature in cardiology for ECOG associations under immunotherapy; early-kill if effect is null or contradictory
- experiment 3 (biomarker correlation): measure ECOG proxy/marker vs clinical endpoints in cardiology cohorts receiving immunotherapy-related interventions; early-kill if correlation < threshold
- experiment 4 (mechanism proxy assay): run a cheap in vitro assay relevant to cardiology with perturbation of ECOG under immunotherapy-like conditions; early-kill if no response

vs control

- experiment 5 (replication stress test): repeat the strongest assay with 2 independent datasets/conditions; early-kill if signal is not reproducible

Target field: neurology

Creep positive papers: 0

Score: 9.0

Proposed experiments:

- experiment 1 (triple-intersection pilot): identify 5 patients with oncology + neurology comorbidity under immunotherapy-relevant treatment context; track pre/post markers; early-kill if no directional change in 2–4 weeks
- experiment 2 (retrospective signal): query existing records/literature in neurology for ECOG associations under immunotherapy; early-kill if effect is null or contradictory
- experiment 3 (biomarker correlation): measure ECOG proxy/marker vs clinical endpoints in neurology cohorts receiving immunotherapy-related interventions; early-kill if correlation < threshold
- experiment 4 (mechanism proxy assay): run a cheap in vitro assay relevant to neurology with perturbation of ECOG under immunotherapy-like conditions; early-kill if no response vs control
- experiment 5 (replication stress test): repeat the strongest assay with 2 independent datasets/conditions; early-kill if signal is not reproducible

Target field: metabolic

Creep positive papers: 1

Score: 4.5

Proposed experiments:

- experiment 1 (triple-intersection pilot): identify 5 patients with oncology + metabolic comorbidity under immunotherapy-relevant treatment context; track pre/post markers; early-kill if no directional change in 2–4 weeks
- experiment 2 (retrospective signal): query existing records/literature in metabolic for ECOG associations under immunotherapy; early-kill if effect is null or contradictory
- experiment 3 (biomarker correlation): measure ECOG proxy/marker vs clinical endpoints in metabolic cohorts receiving immunotherapy-related interventions; early-kill if correlation < threshold
- experiment 4 (mechanism proxy assay): run a cheap in vitro assay relevant to metabolic with perturbation of ECOG under immunotherapy-like conditions; early-kill if no response vs control
- experiment 5 (replication stress test): repeat the strongest assay with 2 independent datasets/conditions; early-kill if signal is not reproducible

Target field: pulmonology

Creep positive papers: 0

Score: 9.0

Proposed experiments:

- experiment 1 (triple-intersection pilot): identify 5 patients with oncology + pulmonology comorbidity under immunotherapy-relevant treatment context; track pre/post markers; early-kill if no directional change in 2–4 weeks
- experiment 2 (retrospective signal): query existing records/literature in pulmonology for ECOG associations under immunotherapy; early-kill if effect is null or contradictory
- experiment 3 (biomarker correlation): measure ECOG proxy/marker vs clinical endpoints in pulmonology cohorts receiving immunotherapy-related interventions; early-kill if correlation < threshold
- experiment 4 (mechanism proxy assay): run a cheap in vitro assay relevant to pulmonology with perturbation of ECOG under immunotherapy-like conditions; early-kill if no response vs control
- experiment 5 (replication stress test): repeat the strongest assay with 2 independent datasets/conditions; early-kill if signal is not reproducible

Target field: immunology

Creep positive papers: 0

Score: 9.0

Proposed experiments:

- experiment 1 (triple-intersection pilot): identify 5 patients with oncology + immunology comorbidity under immunotherapy-relevant treatment context; track pre/post markers; early-kill if no directional change in 2–4 weeks
- experiment 2 (retrospective signal): query existing records/literature in immunology for ECOG associations under immunotherapy; early-kill if effect is null or contradictory
- experiment 3 (biomarker correlation): measure ECOG proxy/marker vs clinical endpoints in immunology cohorts receiving immunotherapy-related interventions; early-kill if correlation < threshold
- experiment 4 (mechanism proxy assay): run a cheap in vitro assay relevant to immunology with perturbation of ECOG under immunotherapy-like conditions; early-kill if no response vs control
- experiment 5 (replication stress test): repeat the strongest assay with 2 independent datasets/conditions; early-kill if signal is not reproducible